

ENGLISH FOR WORK / SEPTEMBER 2026

Nursing and Allied Health English



Learner workbook

Work with realistic cases, develop your own response, and use the answer section after you have tried the tasks.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Shift Handoffs and Clinical Prioritization

Give a handoff that the receiving person can confirm.

02 Patient Assessment and Escalation

Make a request with a clear action, purpose, and realistic timing.

03 Medication Safety and Allergy Clarification

Clarify missing information before responding.

04 Patient Education and Teach-Back

Explain a useful distinction in plain English.

05 Interprofessional Rounds

Include relevant voices and clarify the decision process.

06 Documentation and Charting

Separate observations, assumptions, and conclusions.

07 Difficult Families and Boundaries

Acknowledge a communication problem and give a corrected message.

08 Safety Events and Just Culture

Separate observations, assumptions, and conclusions.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop, or use the prompts at the end of this guide.

LESSON 01 / READ AND NOTICE

Shift Handoffs and Clinical Prioritization

Give a handoff that the receiving person can confirm.

SITUATION

In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

SBAR - Situation, Background, Assessment, and Recommendation or Request: a framework for organizing a clear handoff or concern.

acuity - The severity or complexity of a patient's condition and associated care needs.

pending lab - A requested laboratory test or result that is not yet available.

watcher - A local clinical label sometimes used for a patient needing closer attention; its criteria must be clarified locally.

3. Notice the language

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

Compare this wording: Someone should follow up on this.

LESSON 01 / PRACTICE AND PRODUCE

Transfer information and responsibility

4. Check the language

1. Which sentence checks that responsibility has been accepted?
 - A. The check appears somewhere in the notes.
 - B. Please confirm who will complete the outstanding check.
 - C. I mentioned the check in my earlier email.
2. Which response best checks a critical handoff detail?
 - A. So the result is pending and your team will follow up; is that correct?
 - B. I assume all results are final, so we can finish.
 - C. I saw the message, so no clarification is needed.

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

An SBAR handoff with a pending result

Fictional clinical simulation: case S4 reported tiredness at 14:00 and answered questions while seated. A blood count requested at 13:00 has no result displayed; sample receipt is unknown. Diagnosis and treatment remain outside this language exercise.

01 • Outgoing nurse: Using SBAR for S4: at fourteen hundred, the patient reported tiredness and answered my questions while seated.

02 • Incoming nurse: What background and pending information do I need before accepting the handoff?

03 • Outgoing nurse: A blood count was requested at thirteen hundred. No result is displayed; I haven't confirmed sample receipt.

04 • Incoming nurse: So pending lab doesn't mean you've verified that the laboratory has the sample?

05 • Outgoing nurse: Correct. I observed the conversation, but I haven't established the cause of the reported tiredness.

06 • Incoming nurse: What follow-up are you asking me to take ownership of at this handoff?

07 • Outgoing nurse: Please verify the sample and result status through our clinical process and confirm the appropriate follow-up.

08 • Incoming nurse: I'll own that check; tiredness was reported, questions were answered, and the blood-count status remains unresolved.

09 • Outgoing nurse: That's an accurate read-back. I haven't inferred a diagnosis from the pending result.

10 • Incoming nurse: Handoff accepted. I'll communicate the verified status and follow the local process for concerns.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

Clarifying the watcher label at shift change

Separate fictional handoff exercise: the training board labels case W3 a watcher, but the reason and current review status are not included.

01 • Receiving nurse: Case W3 is labeled watcher. What concern does that label communicate in this simulation?

02 • Shift coordinator: The board doesn't include the reason, so I need to verify it with the responsible clinician.

03 • Receiving nurse: I don't want to infer acuity or a monitoring plan from the label alone.

04 • Shift coordinator: Agreed. We need the actual concern and the current plan, not just shorthand.

05 • Receiving nurse: Could you obtain that clarification before responsibility is treated as fully transferred?

06 • Shift coordinator: Yes. I'll remain responsible for resolving this handoff gap and identify the responding clinician.

07 • Receiving nurse: Please keep any pending lab or review separate from completed assessments in the update.

08 • Shift coordinator: I will. Nothing in the board entry confirms that those activities have happened.

09 • Receiving nurse: I'll review the information once you return and read back the assigned follow-up.

10 • Shift coordinator: Good. We'll close the loop with a verified concern and owner, not an assumed meaning for watcher.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

A mobility handoff with an assessment still pending

Separate fictional allied-health simulation: a physiotherapy assessment has been requested but not completed; no mobility instructions are supplied.

01 • Physiotherapy assistant: The assessment request is in the queue, but the physiotherapist hasn't completed the evaluation.

02 • Receiving nurse: Thank you. I had heard that physiotherapy had already cleared the simulated patient.

03 • Physiotherapy assistant: That isn't the status I can confirm. A request is not a completed assessment.

04 • Receiving nurse: Then I shouldn't infer functional status or change any plan from that message.

05 • Physiotherapy assistant: Correct. Please use the existing authorized plan and direct clinical questions to the responsible clinician.

06 • Receiving nurse: Who will communicate the assessment outcome when it becomes available in the exercise?

07 • Physiotherapy assistant: I'll confirm the assigned physiotherapist and ask them to provide the clinical handoff.

08 • Receiving nurse: I'll keep the assessment marked pending and make that clear to the next shift.

09 • Physiotherapy assistant: Please also correct the earlier clearance message so it doesn't continue circulating.

10 • Receiving nurse: I will. I'll distinguish the requested assessment, its eventual completion, and any separately confirmed clinical recommendations.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Shift Handoffs and Clinical Prioritization

In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Second round: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for scenario 1: The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

Scenario 2 | An owner missing from a pending review

A simulated handoff lists an occupational-therapy review as pending. The outgoing clinician knows it was requested but has not confirmed who accepted it. Practice transferring the information while resolving ownership; no clinical findings are supplied.

Role A: Outgoing nurse: identify the pending review and missing owner.

Role B: Incoming occupational therapist: ask for the request reference and agree how acceptance will be confirmed.

Possible opening: The review was requested, but I cannot yet confirm which clinician accepted it.

Success checks

- Distinguish a request from accepted responsibility.
- Do not invent findings or a clinical plan.
- Read back the agreed ownership check and follow-up route.

Add a complication: The next shift assumes pending means someone is already following it up.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 02 / READ AND NOTICE

Patient Assessment and Escalation

Make a request with a clear action, purpose, and realistic timing.

SITUATION

During a simulation, a patient reports feeling worse and the learner identifies a change from the earlier observation. The local escalation process is available.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

vital signs - Core clinical measurements such as temperature, pulse, respiratory rate, blood pressure, and oxygen saturation used to assess patient status.

rapid response - Clinical team activated to assess and stabilize a patient showing signs of acute deterioration before a full emergency event.

clinical deterioration - Worsening patient condition that requires prompt reassessment, communication, and possible escalation of care.

escalation - Raising an issue to a higher authority or different function because risk, urgency, or decision rights require it.

3. Notice the language

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

Compare this wording: Please revert soonest with the necessary.

LESSON 02 / PRACTICE AND PRODUCE

Ask for an actionable next step

4. Check the language

1. What does 'Please send it by Thursday' normally mean?
 - A. Send it no later than Thursday.
 - B. Send it on Thursday, but not earlier.
 - C. Keep sending it regularly until Thursday.
2. Which request gives the recipient enough detail to act?
 - A. Could you confirm that the review is important?
 - B. Could you confirm that you received my earlier message?
 - C. Could you confirm the document version needed for the review?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

Requesting review for a reported change

Fictional clinical simulation: at 10:00 case C2 spoke in full sentences; at 10:20 the patient said I feel worse and paused repeatedly while answering. No vital-sign values, diagnosis, or treatment instruction is supplied; the local escalation process is available.

01 • Bedside nurse: I'm requesting review of C2 now. At ten twenty, the patient said, I feel worse.

02 • Charge nurse: What did you observe, separately from the patient's own description of the change?

03 • Bedside nurse: At ten, they spoke in full sentences. At ten twenty, they paused repeatedly while answering.

04 • Charge nurse: Do you have vital-sign readings or a confirmed explanation for that change?

05 • Bedside nurse: Neither is supplied here. I'm reporting the change, not assigning a diagnosis.

06 • Charge nurse: I'll receive the concern and use our local escalation process to reach the responsible clinician.

07 • Bedside nurse: Please confirm who accepts the review request; I don't want the contact left unacknowledged.

08 • Charge nurse: Understood: worsening reported and repeated pauses observed at ten twenty, compared with full sentences at ten.

09 • Bedside nurse: Correct. No review outcome or rapid-response activation has been confirmed in this conversation.

10 • Charge nurse: I'll coordinate the next contact through the local process and confirm the receiving clinician.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

A callback without a confirmed receiver

Separate fictional escalation rehearsal: a nurse left a message about a change, but no clinician has acknowledged receipt; local escalation instructions are available.

01 • Simulation nurse: I left a message about the change, but no one has acknowledged receiving it.

02 • Clinical supervisor: Then the communication loop is still open. What does the local escalation process direct next?

03 • Simulation nurse: I'll check and follow that process instead of assuming the message has been acted on.

04 • Clinical supervisor: Make your request explicit when you reach the next responsible person.

05 • Simulation nurse: I need a clinician to review the reported change and confirm that they have accepted the request.

06 • Clinical supervisor: Yes. Include the actual observations and timing supplied in the exercise, without adding findings.

07 • Simulation nurse: Could I describe the lack of acknowledgment as proof that the clinician ignored me?

08 • Clinical supervisor: No. It establishes an unconfirmed response, not the person's intention or what they heard.

09 • Simulation nurse: I'll restate the concern through the escalation route and ask for a clear acknowledgment.

10 • Clinical supervisor: Good. We'll record the contacts and verified response while keeping the clinical concern visible.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

An unexpected monitor value needs a clear review request

Separate fictional measurement simulation: the training monitor displayed a pulse of 78 beats per minute at 09:00 and 108 at 09:20. Measurement reliability and clinical significance are unassessed; these numbers are not action thresholds or treatment guidance.

01 • Allied health clinician: I'm requesting a nursing review: the training monitor's displayed pulse changed between nine and nine twenty.

02 • Registered nurse: Please give the exact readings and distinguish the display from a confirmed clinical finding.

03 • Allied health clinician: It displayed seventy-eight beats per minute at nine and one hundred eight at nine twenty.

04 • Registered nurse: Have you established whether that reflects the patient or an issue with measurement quality?

05 • Allied health clinician: No. The explanation is unconfirmed; I haven't dismissed it as equipment error or diagnosed deterioration.

06 • Registered nurse: I'll assess the concern within my role and follow our local clinical process.

07 • Allied health clinician: Could you read back the values so I can confirm that I communicated them clearly?

08 • Registered nurse: Seventy-eight at nine, one hundred eight at nine twenty; reliability and significance still need assessment.

09 • Allied health clinician: Correct. Those are displayed readings, and I'm asking for review rather than prescribing a response.

10 • Registered nurse: Request received. I'll confirm the appropriate follow-up through the clinical process.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Assessment and Escalation

During a simulation, a patient reports feeling worse and the learner identifies a change from the earlier observation. The local escalation process is available.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Second round: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Model for scenario 1: I'm concerned because this is a change from the earlier observation. I need an assessment through our escalation process now. Here is what I observed and when it changed.

Scenario 2 | A concern interrupted during a call

In a clinical simulation, a learner starts reporting a change from baseline, but the call is interrupted before the request is acknowledged. No review has been confirmed. Practice reopening the call and stating the action needed using the local escalation route.

Role A: Bedside clinician: restate the observed concern and explicit review request.

Role B: Receiving charge nurse: confirm what you heard and clarify responsibility.

Possible opening: Our call ended before I confirmed that my request for review had been received.

Success checks

- State that review is requested, not merely that information exists.
- Use only supplied observations and avoid diagnosing the cause.
- Obtain acknowledgment and follow the local process for unresolved concerns.

Add a complication: The receiver initially thinks you are calling only with a routine status update.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 03 / READ AND NOTICE

Medication Safety and Allergy Clarification

Clarify missing information before responding.

SITUATION

A fictional medication record and the patient's account contain different allergy information. The learner must communicate the discrepancy through the clinical process.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

allergy - An immune response to a substance that can cause symptoms of varying severity.

contraindication - A circumstance that makes a treatment or procedure inadvisable because of potential harm.

MAR - Medication administration record: the clinical record used to document medication administration information.

closed-loop communication - A message-and-confirmation process in which the receiver repeats information and the sender verifies understanding.

3. Notice the language

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Compare this wording: Could you explain what does this mean?

LESSON 03 / PRACTICE AND PRODUCE

Ask a precise question

4. Check the language

1. Which indirect question has the usual word order?
 - A. Could you confirm when does the review start?
 - B. Could you confirm when starts the review?
 - C. Could you confirm when the review starts?
2. Someone says the report is 'ready.' Which question best clarifies its approval status?
 - A. Who will present the report at the meeting?
 - B. Has the report been approved, or is it ready for review?
 - C. Can you send the report to me this afternoon?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

An allergy account that differs from the record

Fictional medication-reconciliation simulation: the chart says no known drug allergies. The patient reports a rash after penicillin years ago but cannot recall the date; prior verification and causality are unknown. No medication or treatment decision is supplied.

01 • Registered nurse: The chart says no known drug allergies, but the patient reports a rash after penicillin years ago.

02 • Clinical pharmacist: Did the patient give a date, and has that reported reaction been assessed previously?

03 • Registered nurse: They couldn't recall the date. I don't know whether either source was previously verified.

04 • Clinical pharmacist: Then preserve the reported rash and the chart entry separately; neither resolves the discrepancy by itself.

05 • Registered nurse: Should I describe penicillin as a confirmed cause of the rash in the handoff?

06 • Clinical pharmacist: No. Say the patient reported a rash after it; causality remains unassessed here.

07 • Registered nurse: I'm requesting reconciliation through our clinical process, not proposing a change to the MAR myself.

08 • Clinical pharmacist: Request received: charted negative history, reported penicillin-associated rash, date unknown, verification pending.

09 • Registered nurse: That's accurate. Please confirm the responsible review and communicate any authorized clarification back to the team.

10 • Clinical pharmacist: I will coordinate that follow-up and keep the allergy discrepancy visible until it is reviewed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

Unknown is not the same as no known allergies

Separate fictional chart review: an imported allergy field is blank; no patient interview or reconciliation has been completed.

01 • Medication reconciliation nurse: The imported allergy field is blank. Can I read that as no known allergies?

02 • Pharmacist: No. In this case, a blank field doesn't establish that an allergy history was checked.

03 • Medication reconciliation nurse: So the correct communication is allergy status unknown, pending the appropriate clarification?

04 • Pharmacist: Yes. State what is missing and who needs to verify it through the clinical process.

05 • Medication reconciliation nurse: Could the imported MAR help, or would that automatically resolve the uncertainty?

06 • Pharmacist: It may provide context, but an imported medication record isn't proof that allergy reconciliation occurred.

07 • Medication reconciliation nurse: I'll ask the responsible clinician to confirm the source and status rather than fill the gap myself.

08 • Pharmacist: Please use closed-loop communication so the clarification request is acknowledged.

09 • Medication reconciliation nurse: I'll read back the unresolved item and confirm the receiving clinician's responsibility for follow-up.

10 • Pharmacist: Good. Keep unknown information distinct from a verified negative history throughout the simulation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

A discussion mistaken for a medication instruction

Separate fictional medication-communication rehearsal: a colleague discussed a possible contraindication, but no reviewed instruction has been issued.

01 • Ward nurse: I heard a discussion about a possible contraindication. Was that a confirmed medication instruction?

02 • Clinical pharmacist: Not in the information supplied here. A discussion and an authorized instruction are different things.

03 • Ward nurse: Could you clarify who is reviewing the concern and where the confirmed instruction will appear?

04 • Clinical pharmacist: I'll verify the responsible reviewer through the established clinical process rather than invent an owner.

05 • Ward nurse: I don't want an informal remark to become a change in the MAR.

06 • Clinical pharmacist: Agreed. Preserve the concern, but don't represent it as a completed assessment or approved change.

07 • Ward nurse: What should I say in the meantime when another nurse asks about the conversation?

08 • Clinical pharmacist: Say a possible contraindication was raised and the reviewed instruction has not been confirmed.

09 • Ward nurse: I'll use that wording and request acknowledgment when the authorized clarification is available.

10 • Clinical pharmacist: I'll help close the loop so the team receives a verified message rather than a chain of assumptions.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Medication Safety and Allergy Clarification

A fictional medication record and the patient's account contain different allergy information. The learner must communicate the discrepancy through the clinical process.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Second round: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for scenario 1: The recorded allergy information differs from what the patient has told me. Could we verify the details through the appropriate clinical process? I will state the discrepancy clearly rather than assume either account is complete.

Scenario 2 | An unreadable allergy entry

In a fictional charting simulation, an allergy entry contains an unreadable abbreviation and has not been reconciled. No substance or reaction is confirmed. Practice requesting clarification without guessing the abbreviation or making a medication decision.

Role A: Registered nurse: identify the exact unreadable entry and request source verification.

Role B: Clinical pharmacist: clarify what remains unknown and arrange the appropriate review.

Possible opening: I cannot reliably interpret this allergy abbreviation, so I need the source entry clarified.

Success checks

- Do not substitute a guessed substance or reaction.
- Distinguish the unclear entry from a confirmed allergy finding.
- Confirm the review route and use a read-back when verified information arrives.

Add a complication: A colleague offers a likely expansion from memory and asks you to use it.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 04 / READ AND NOTICE

Patient Education and Teach-Back

Explain a useful distinction in plain English.

SITUATION

A patient in a role-play nods during an explanation of an approved discharge sheet. The learner needs to check whether the explanation was clear.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

teach-back - Asking a person to explain information in their own words to check how clearly it was communicated.

discharge instructions - Information given to support a patient's care and follow-up after leaving a care setting.

health literacy - The ability to find, understand, and use information and services for health-related decisions.

adherence - The extent to which a person's actions follow an agreed care or treatment plan.

3. Notice the language

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Compare this wording: We need to operationalize the implementation.

LESSON 04 / PRACTICE AND PRODUCE

Make a complex point clear

4. Check the language

1. Which sentence explains 'backlog' rather than repeating it?
 - A. The backlog is the maximum work the team can finish in a period.
 - B. The backlog is the work still waiting to be completed.
 - C. The backlog is the work completed during the last period.
2. Which follow-up best checks whether your explanation was clear?
 - A. How would you describe the next step in your own words?
 - B. Would you like me to repeat the explanation?
 - C. Was the explanation detailed enough for you?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

Teach-back after a nod

Fictional education rehearsal: the approved administrative sheet directs appointment-change requests to the Scheduling Office, not the ward desk. A standardized patient has nodded during the explanation. No clinical advice is practiced.

01 • Discharge nurse: This sheet directs appointment-change requests to the Scheduling Office rather than the ward desk.

02 • Standardized patient: Yes, I see the two office names. I think I understand what you're saying.

03 • Discharge nurse: To check how clearly I explained it, whom would you contact to request a different appointment?

04 • Standardized patient: I'd call the ward desk and ask them to move the appointment for me.

05 • Discharge nurse: I wasn't clear enough: for an appointment change, contact the Scheduling Office, as shown here.

06 • Standardized patient: So the ward desk isn't the contact for that request on this sheet?

07 • Discharge nurse: Correct. Could you explain the appointment-change step again in your own words?

08 • Standardized patient: If I need another appointment time, I'll contact the Scheduling Office to request a change.

09 • Discharge nurse: Exactly. You've identified the right office; a request doesn't itself confirm a new appointment.

10 • Standardized patient: Understood. I'll request the change there and wait for confirmation of any new time.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

Repeating the words versus explaining the next step

Separate fictional teach-back simulation: an approved administrative sheet says to collect an attendance certificate from Reception after check-in, not before. The standardized patient can repeat the heading but has not explained the sequence.

01 • Standardized patient: I remember the heading: attendance certificate collection. Does that mean I should collect it first?

02 • Discharge nurse: The sheet says to check in first, then collect the certificate from Reception.

03 • Standardized patient: I can repeat those words, but I was unsure whether collection comes before check-in.

04 • Discharge nurse: Thank you for checking. Repeating a heading and understanding the sequence aren't necessarily the same.

05 • Standardized patient: Could you ask me a practical question so I can check that I've understood?

06 • Discharge nurse: Certainly. When you arrive, what will you do before collecting your attendance certificate?

07 • Standardized patient: I'll check in first, then go to Reception to collect the certificate.

08 • Discharge nurse: That's right. You have both the order and the collection place correct.

09 • Standardized patient: So I won't go straight to certificate collection before completing check-in.

10 • Discharge nurse: Exactly. That matches this sheet, and I'm glad we clarified the sequence together.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

An unfamiliar word on the discharge sheet

Separate fictional allied-health education rehearsal: the approved administrative sheet lists a follow-up appointment for Tuesday at 09:00, with check-in at Reception. The standardized patient does not understand follow-up; no clinical instruction is practiced.

01 • Occupational therapist: Which words on this approved sheet would you like me to explain before we finish?

02 • Standardized patient: I am unsure about follow-up. Does it mean I should arrive today without an appointment?

03 • Occupational therapist: It means a later check or contact. Your listed appointment is Tuesday at nine, with check-in at Reception.

04 • Standardized patient: So follow-up is not an instruction to come immediately; the appointment details tell me when?

05 • Occupational therapist: Correct. To check my explanation, could you tell me when and where you will check in?

06 • Standardized patient: I will check in at Reception on Tuesday at nine for the listed appointment.

07 • Occupational therapist: Exactly. That matches the time and place on your sheet, and you explained it clearly.

08 • Standardized patient: If another detail is unclear, should I guess rather than ask the team again?

09 • Occupational therapist: Please ask for clarification. We can identify the right contact without inventing an answer.

10 • Standardized patient: Thank you. I understand the appointment arrangement and will ask about anything still unclear.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Education and Teach-Back

A patient in a role-play nods during an explanation of an approved discharge sheet. The learner needs to check whether the explanation was clear.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Second round: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Model for scenario 1: I want to check that I explained this clearly. In your own words, how will you follow the instructions when you get home? We can go over any part that is unclear.

Scenario 2 | A translation request before teach-back

In a fictional education exercise, the standardized patient says the approved discharge sheet is in a language they cannot read comfortably. An appropriate language-support request route is available, but support has not yet been arranged.

Role A: Discharge nurse: acknowledge the language need and request appropriate support without improvising clinical translation.

Role B: Interpreter coordinator: clarify the requested language and explain that availability still needs confirmation.

Possible opening: Before checking understanding, I need to arrange appropriate language support for this explanation.

Success checks

- Distinguish language access from demonstrated understanding.
- Do not invent interpreter availability or translate unsupplied clinical content.
- Agree the support request and a later teach-back check using approved information.

Add a complication: A colleague suggests treating the patient's polite nod as sufficient understanding.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 05 / READ AND NOTICE

Interprofessional Rounds

Include relevant voices and clarify the decision process.

SITUATION

In simulated rounds, one team member focuses on discharge timing and another raises an unresolved mobility concern.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

plan of care - A coordinated record of a patient's assessed needs, goals, and planned care.

functional status - A person's ability to perform activities relevant to daily life and care planning.

case management - Coordinating services and support around an individual's needs and circumstances.

safe discharge - A care transition planned to address the patient's identified needs through the applicable clinical process.

3. Notice the language

Summarize the issue neutrally, invite missing perspectives, and distinguish discussion from decision. State who can decide and record any unresolved question. Do not assume silence means agreement.

Let's hear ... before we decide.

The point we still need to resolve is

Who will make this decision, and what do they need?

Compare this wording: Nobody objected, so everyone agrees.

LESSON 05 / PRACTICE AND PRODUCE

Help a group reach a clear next step

4. Check the language

1. Which phrase invites a missing perspective?
 - A. We haven't heard from the remote team yet; what is your view?
 - B. The remote team has been quiet, so they must agree.
 - C. We can record their agreement without asking them.
2. Which closing statement records an unresolved decision accurately?
 - A. We discussed the options, so approval is complete.
 - B. Everyone spoke, so the decision must be final.
 - C. Approval is pending; the sponsor will review the two options.

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Making room for the mobility concern

Fictional interprofessional rounds: discharge timing is being discussed, while a mobility concern remains unresolved.

01 • Rounds coordinator: Before we discuss a discharge time, let's hear the unresolved mobility concern from physiotherapy.

02 • Physiotherapist: Thank you. The concern needs assessment, and I don't want the timing discussion to imply clearance.

03 • Rounds coordinator: What information does the team need before the responsible clinician considers the plan of care?

04 • Physiotherapist: The current functional-status assessment and any remaining questions, without assuming the outcome in advance.

05 • Rounds coordinator: Case management needs timing information too. Can we identify that dependency without deciding discharge here?

06 • Physiotherapist: Yes. Say timing remains dependent on the relevant clinical review rather than promise a departure.

07 • Rounds coordinator: I'll invite case management to explain its coordination needs after your clinical concern is heard.

08 • Physiotherapist: Please also clarify who has authority to confirm the discharge plan in this simulation.

09 • Rounds coordinator: I'll verify that role and summarize the unresolved concern before seeking a decision.

10 • Physiotherapist: Good. The group can coordinate a safe discharge discussion without treating silence or schedule pressure as clinical agreement.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

Equipment requested but not confirmed

Separate fictional rounds exercise: an occupational-therapy equipment request is pending, while case management is preparing discharge logistics.

01 • Case manager: I'm organizing the logistics, but I need to know whether the equipment request is confirmed.

02 • Occupational therapist: The request is pending. We haven't received confirmation of availability or the final arrangement.

03 • Case manager: Could I list equipment ready and let the receiving team check it later?

04 • Occupational therapist: That would hide the unresolved dependency. Please keep requested and confirmed as separate statuses.

05 • Case manager: Let's bring the responsible supply coordinator into the discussion before we settle the logistics.

06 • Occupational therapist: Agreed. I'll explain the assessment-related request without implying a new clinical recommendation here.

07 • Case manager: Who should confirm the final plan of care once the relevant information is available?

08 • Occupational therapist: The authorized clinical team; our logistical discussion doesn't replace that decision.

09 • Case manager: I'll summarize the pending equipment question and assign the availability check for confirmation.

10 • Occupational therapist: I'll review the returned information with the team so we can distinguish coordination progress from clinical readiness.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

Different meanings of ready during rounds

Separate fictional rounds rehearsal: nursing says education is ready to deliver, while the meeting chair interprets ready as all discharge work completed.

01 • Meeting chair: You said ready. Are all parts of the simulated discharge plan complete now?

02 • Registered nurse: I meant the education materials are ready to deliver, not that education or discharge is complete.

03 • Meeting chair: Thank you for correcting that. Which other voices should we hear before discussing readiness?

04 • Registered nurse: Ask each relevant discipline to state its own status instead of letting my comment stand for the team.

05 • Meeting chair: I'll invite case management and therapy to identify completed work and unresolved dependencies.

06 • Registered nurse: Please separate information sharing from the authorized decision about the plan of care.

07 • Meeting chair: Agreed. I won't record team agreement merely because nobody interrupted the word ready.

08 • Registered nurse: I can report the education step accurately after it occurs and its understanding check is completed.

09 • Meeting chair: For now, I'll summarize materials prepared, education pending, and other disciplines' status awaiting confirmation.

10 • Registered nurse: That gives the team a clear starting point without turning one completed preparation task into a discharge decision.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Interprofessional Rounds

In simulated rounds, one team member focuses on discharge timing and another raises an unresolved mobility concern.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You hold a relevant concern that has not been discussed. Raise it when invited and ask who will decide. Help the facilitator state one unresolved question accurately.

Second round: A participant interrupts or the meeting is running out of time. Protect a turn to speak and close with an accurate decision record.

Model for scenario 1: Before we agree on timing, could we hear the mobility concern and the current assessment? Let's confirm the outstanding questions and who will address them in the care plan.

Scenario 2 | A missing discipline at the huddle

In a simulated discharge huddle, the speech and language therapist is absent and their review is still pending. The group wants to finalize coordination arrangements. No assessment outcome is supplied. Facilitate a bounded discussion of the missing input.

Role A: Rounds facilitator: invite available perspectives and identify the missing review.

Role B: Case manager: explain logistical needs without substituting for the absent clinician.

Possible opening: We can discuss coordination needs, but the pending review must remain visible before an authorized decision.

Success checks

- Reject silence as evidence of clinical agreement.
- Separate logistics from the missing clinical assessment.
- Assign a contact owner and clarify who can confirm the plan.

Add a complication: A participant says the absent therapist's silence means agreement.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 06 / READ AND NOTICE

Documentation and Charting

Separate observations, assumptions, and conclusions.

SITUATION

A draft simulated note says a patient was "difficult." It contains no description of what was said or observed.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

charting - Recording patient information and care in the clinical record.

objective finding - An observation or measured result described without unsupported judgment about intent.

intervention - An action intended to change a situation or improve an outcome.

patient response - A recorded change, statement, or reaction following care or an intervention.

3. Notice the language

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

The available evidence shows

This may indicate ..., but

We have not yet established whether

Compare this wording: The pilot proves this will work everywhere.

LESSON 06 / PRACTICE AND PRODUCE

Match certainty to evidence

4. Check the language

1. Which sentence preserves uncertainty about an unconfirmed cause?
 - A. The change must have caused the delay.
 - B. The change definitely caused the delay.
 - C. The change may have contributed to the delay.
2. A result comes from a small pilot in one location. Which phrase accurately limits the claim?
 - A. Across the service, the observed result was ...
 - B. In this pilot, the observed result was ...
 - C. In similar locations, we can now expect the same result ...

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

Replace difficult with an observed account

Fictional charting exercise: a draft note says difficult but includes no observed behavior or patient quotation.

01 • Clinical preceptor: This simulated note calls the patient difficult. What did you actually observe or hear?

02 • New nurse: The draft doesn't include that information, so I can't support the label with an objective finding.

03 • Clinical preceptor: Could you replace it with uncooperative and keep the rest of the note?

04 • New nurse: That would still be an interpretation without a described event or patient response.

05 • Clinical preceptor: What do you need before revising the charting exercise accurately and usefully?

06 • New nurse: The actual observation, relevant words, and any documented intervention, without inventing a behavioral example.

07 • Clinical preceptor: Good. A neutral description should help the reader understand the event rather than judge the person.

08 • New nurse: I'll ask for the missing source details and distinguish quotations from my own observations.

09 • Clinical preceptor: Also keep the effect of any intervention separate from what you hoped it would achieve.

10 • New nurse: Understood. I won't replace an unsupported label with a fabricated fact just to make the note look complete.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

A request for more explanation is not refusal

Separate fictional documentation simulation: the patient said, 'Please explain that again before we continue'; no further response is supplied.

01 • Staff nurse: The simulated patient asked me to explain again before continuing. Can I chart refused?

02 • Nurse educator: That wording goes beyond the supplied statement. They requested another explanation, not necessarily refused.

03 • Staff nurse: Would quoting the actual sentence preserve the distinction more accurately in the record?

04 • Nurse educator: Yes. Identify it as the patient's statement and describe any actual subsequent action separately.

05 • Staff nurse: The exercise doesn't say whether I explained again or whether the patient then agreed.

06 • Nurse educator: Then neither outcome should appear as a completed intervention or patient response.

07 • Staff nurse: I'll keep the unknown sequence visible rather than supply a likely ending for the chart.

08 • Nurse educator: Good. Objective charting can contain uncertainty when the source information is incomplete.

09 • Staff nurse: I'll ask for the remaining simulation details before documenting anything beyond the quotation.

10 • Nurse educator: That preserves what was said without turning a communication request into an unsupported judgment about cooperation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

Observed later does not mean caused by the intervention

Separate fictional note review: an intervention is documented and a later patient response is recorded, but the relationship has not been assessed.

01 • Allied health clinician: My note says the intervention caused the later improvement. Is that supported by the sequence alone?

02 • Documentation mentor: The sequence establishes that one happened before the other, not necessarily why the response changed.

03 • Allied health clinician: Should I remove the patient response completely to avoid making a causal claim?

04 • Documentation mentor: No. Preserve the observed response and its timing, while qualifying any interpretation of its cause.

05 • Allied health clinician: The simulation gives no additional assessment, so I shouldn't invent a clinical explanation.

06 • Documentation mentor: Correct. Distinguish the recorded intervention, the objective finding, and any assessment actually made.

07 • Allied health clinician: Could I say the response was observed after the intervention without saying caused by?

08 • Documentation mentor: That is a more accurate temporal description, provided it matches the supplied record.

09 • Allied health clinician: I'll revise the causal wording and retain the source observations for the reader.

10 • Documentation mentor: Good. The note can be useful without claiming a relationship that this exercise hasn't established.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Documentation and Charting

A draft simulated note says a patient was "difficult." It contains no description of what was said or observed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: Let's replace that label with observable information. Record what the patient said or did, the relevant context, the action taken, and the response, following the documentation process.

Scenario 2 | A colleague's observation in your note

In a fictional charting simulation, another clinician tells you a patient declined a conversation. You did not witness it, and the exact words are unknown. Discuss how to communicate the source and uncertainty before any documentation is finalized.

Role A: Receiving clinician: distinguish secondhand information from your own observation.

Role B: Reporting clinician: clarify what you actually observed without adding motives.

Possible opening: I did not witness the conversation, so I need to distinguish your report from my own findings.

Success checks

- Attribute the report to its actual source.
- Avoid inventing quotations, emotion, or motivation.
- Agree which observed details need clarification through the documentation process.

Add a complication: A colleague suggests writing the patient was angry, although no behavior supporting that description is supplied.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 07 / READ AND NOTICE

Difficult Families and Boundaries

Acknowledge a communication problem and give a corrected message.

SITUATION

A family member in a role-play says, "Nobody tells us anything." The learner can explain their role and arrange an appropriate conversation.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

scope of practice - The activities a professional is permitted and qualified to perform under applicable rules.

privacy - The appropriate handling of personal information and individuals' related interests and rights.

family meeting - A coordinated discussion with a patient's family or support people about relevant care matters.

boundary - A limit on acceptable conduct, responsibility, access, or scope.

3. Notice the language

Name what was unclear or wrong, acknowledge its effect, and correct it. A brief apology can help, followed by a practical next step. Avoid explaining away the other person's reaction or promising an outcome you cannot control.

My earlier message did not make ... clear.

I'm sorry for The correct information is

Let me check that the revised explanation addresses

Compare this wording: I'm sorry if you failed to understand.

LESSON 07 / PRACTICE AND PRODUCE

Repair a misunderstanding

4. Check the language

1. Which apology takes responsibility for unclear wording?
 - A. I'm sorry we could not meet the original date.
 - B. I'm sorry my earlier wording was unclear.
 - C. I'm sorry the process took longer than expected.
2. What should follow an apology for an incorrect date?
 - A. The corrected date and the current confirmation status.
 - B. A longer explanation that never states the correct date.
 - C. A new guaranteed date even if it is unverified.

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

Responding to nobody tells us anything

Fictional family-communication rehearsal between a nurse and a patient liaison: a family member has said nobody tells us anything; sharing permissions are unconfirmed.

01 • Patient liaison: The family says nobody tells us anything. How will you respond without becoming defensive?

02 • Registered nurse: I'll acknowledge their frustration and explain that I can help arrange an appropriate conversation.

03 • Patient liaison: Would you promise to disclose the full clinical record to repair the communication problem?

04 • Registered nurse: No. I need to respect privacy and confirm what information can appropriately be shared.

05 • Patient liaison: How can you explain that boundary without sounding as though you're refusing to help?

06 • Registered nurse: I can help identify the right clinician and arrange a discussion within the applicable sharing arrangements.

07 • Patient liaison: The family may ask you for a decision outside your scope of practice.

08 • Registered nurse: I'll explain my role clearly and refer that question to the responsible clinician rather than guess.

09 • Patient liaison: Please avoid promising a family meeting time before the participants are available.

10 • Registered nurse: Agreed. I'll take responsibility for requesting the conversation and returning a confirmed arrangement.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

Correcting an unconfirmed family-meeting time

Separate fictional communication case: a nurse said the family meeting would be at two, but clinician availability has not been confirmed.

01 • Nurse coordinator: I told the family two o'clock, but I hadn't confirmed the clinician's availability.

02 • Social worker: We need to correct that promise directly rather than leave them waiting for an unconfirmed meeting.

03 • Nurse coordinator: I'll say I gave a time before confirming it, and I'm sorry for that confusion.

04 • Social worker: Good. What can you offer now without creating another unsupported promise?

05 • Nurse coordinator: I can request a confirmed time and remain the contact for the scheduling update.

06 • Social worker: Should the correction also clarify the purpose of the proposed family meeting?

07 • Nurse coordinator: Yes, within the agreed scope and privacy arrangements, without promising a specific clinical decision.

08 • Social worker: I'll help check who needs to attend and what questions the family wants addressed.

09 • Nurse coordinator: I'll distinguish collecting their questions from guaranteeing that every requested outcome can be provided.

10 • Social worker: Then we'll return with a verified arrangement and a clear account of who will address which concerns.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

An apology without crossing a privacy boundary

Separate fictional liaison call: an earlier message implied any relative could receive updates, but the appropriate sharing permissions have not been verified.

01 • Patient liaison: Our earlier message suggested any relative could receive clinical updates. That needs correction.

02 • Charge nurse: I'll acknowledge the misleading wording and explain that appropriate sharing permissions must be checked.

03 • Patient liaison: Can you make that correction without implying the relative has done something wrong?

04 • Charge nurse: Yes. The problem was our explanation; the check is about information sharing, not a judgment about them.

05 • Patient liaison: What should happen if they ask you to confirm details while the check is pending?

06 • Charge nurse: I'll maintain the privacy boundary and help them reach the appropriate conversation route.

07 • Patient liaison: Could you promise that permission will be granted once the form is reviewed?

08 • Charge nurse: No. Review is not a guaranteed outcome, and I shouldn't imply authority I don't have.

09 • Patient liaison: I'll support the follow-up and make sure the family receives a clear explanation of the next step.

10 • Charge nurse: Thank you. We can repair the message and remain helpful without making an unverified disclosure.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Difficult Families and Boundaries

A family member in a role-play says, "Nobody tells us anything." The learner can explain their role and arrange an appropriate conversation.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are frustrated because an earlier message created an expectation. Explain that expectation. Ask your partner to distinguish the correction from anything still uncertain.

Second round: The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Model for scenario 1: I can hear that the lack of information is frustrating. Let me explain what I can clarify and arrange contact with the appropriate team member for the questions outside my role.

Scenario 2 | A question redirected without explanation

In a fictional family liaison exercise, a relative was transferred between two departments without being told why. Their clinical question remains unanswered, and the responsible clinician has not been identified. Practice a repair conversation between staff.

Role A: Patient liaison: acknowledge the confusing transfers and ask for a named contact route.

Role B: Ward coordinator: clarify your own role and arrange identification of the appropriate clinician.

Possible opening: We transferred the question without explaining the route, so we need to give the family a clearer next step.

Success checks

- Acknowledge the communication failure without blaming the relative.
- Keep the response within role and privacy boundaries.
- Agree who identifies and confirms the appropriate contact.

Add a complication: A colleague proposes answering the clinical question from memory to end the complaint.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 08 / READ AND NOTICE

Safety Events and Just Culture

Separate observations, assumptions, and conclusions.

SITUATION

A simulated safety report includes a fall and an assumption that a colleague was careless. The review has not established the contributing factors.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

fall risk - The possibility that a person may fall, assessed in the relevant care context.

incident report - A structured record of an event, available facts, and relevant follow-up information.

just culture - An approach to learning from safety events that examines systems and behavior while retaining appropriate accountability.

root cause - Underlying reason a problem occurred, not merely the visible symptom.

3. Notice the language

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

The available evidence shows

This may indicate ..., but

We have not yet established whether

Compare this wording: The pilot proves this will work everywhere.

LESSON 08 / PRACTICE AND PRODUCE

Match certainty to evidence

4. Check the language

1. Which sentence preserves uncertainty about an unconfirmed cause?
 - A. The change may have contributed to the delay.
 - B. The change must have caused the delay.
 - C. The change definitely caused the delay.
2. A result comes from a small pilot in one location. Which phrase accurately limits the claim?
 - A. In similar locations, we can now expect the same result ...
 - B. Across the service, the observed result was ...
 - C. In this pilot, the observed result was ...

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

A fall report without a blame conclusion

Fictional safety-report review: a fall is reported, but contributing factors are unestablished; a draft calls a colleague careless.

01 • Safety facilitator: The incident report describes a fall and then calls a colleague careless. What supports that conclusion?

02 • Staff nurse: Nothing in the supplied account establishes carelessness or the contributing factors.

03 • Safety facilitator: What should remain in the report while the review gathers additional information?

04 • Staff nurse: The observed event, available timing, and known actions, with unverified details clearly identified.

05 • Safety facilitator: Would removing the blame word mean we are avoiding accountability for the event?

06 • Staff nurse: No. A just-culture review can examine responsibilities without treating an unsupported accusation as a finding.

07 • Safety facilitator: Should you name a root cause now so the exercise has a definite ending?

08 • Staff nurse: No. We can end with an assigned review step while the cause remains unknown.

09 • Safety facilitator: Please also distinguish any recorded fall-risk information from a conclusion about why this fall happened.

10 • Staff nurse: I will. I'll preserve the available evidence and send the unresolved questions through the incident-review process.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

A missing entry after an intercepted mix-up

Separate fictional safety simulation: a label mix-up was intercepted before use; the check's documentation is missing, and the sequence is not fully established.

01 • Clinical safety reviewer: The mix-up was intercepted before use, but the check isn't documented in the supplied record.

02 • Ward supervisor: Can we conclude that the check was skipped and that this caused the mix-up?

03 • Clinical safety reviewer: No. Missing documentation and a demonstrated causal sequence are different findings.

04 • Ward supervisor: What questions should we ask without leading colleagues toward a preferred explanation?

05 • Clinical safety reviewer: Ask what each person saw, what actions occurred, and which records can establish the sequence.

06 • Ward supervisor: Should we omit the event because the interception prevented the intended use?

07 • Clinical safety reviewer: No. Follow the simulation's reporting process; an intercepted event can still provide useful safety information.

08 • Ward supervisor: I'll describe the known interception and keep the undocumented step marked unresolved.

09 • Clinical safety reviewer: I'll gather the available accounts without treating a single explanation as the root cause.

10 • Ward supervisor: Then our discussion can identify follow-up responsibly without claiming either blame or complete system safety.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

One refresher session does not prove prevention

Separate fictional safety review: staff attended a refresher after an incident; no effectiveness assessment or root-cause finding has been completed.

01 • Education coordinator: Everyone attended the refresher. Can we report that the incident's root cause is resolved?

02 • Safety lead: No. Attendance confirms participation, while the cause and effectiveness assessment remain open.

03 • Education coordinator: The session covered fall-risk communication. Isn't that enough to show future falls are prevented?

04 • Safety lead: It doesn't establish that outcome. We shouldn't turn a training activity into a universal prevention claim.

05 • Education coordinator: How do we give a positive but accurate update on what the team completed?

06 • Safety lead: State that the refresher occurred and distinguish it from the pending review and effectiveness work.

07 • Education coordinator: Could staff interpret that distinction as saying their participation didn't matter at all?

08 • Safety lead: Explain its purpose, while being clear about what evidence it does and doesn't provide.

09 • Education coordinator: I'll describe the completed session and keep the incident report linked to the ongoing review.

10 • Safety lead: Good. We'll evaluate the relevant evidence before closing actions or claiming the contributing factors have been addressed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Safety Events and Just Culture

A simulated safety report includes a fall and an assumption that a colleague was careless. The review has not established the contributing factors.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: The report should describe the event and available observations. We have not established the contributing factors. I'll remove the unsupported judgment and use the reporting process to raise the concern.

Scenario 2 | Two accounts of the same safety event

In a fictional incident review, two clinicians remember a call occurring at different times. Neither timestamp has been verified, and no causal conclusion exists. Discuss how to preserve both accounts and seek corroboration.

Role A: Safety interviewer: describe the disagreement neutrally and request available records.

Role B: Clinical supervisor: separate recollection from verified timing and keep the review open.

Possible opening: The accounts differ on the call time, so we need to preserve both while checking the records.

Success checks

- Do not treat seniority as proof that an account is correct.
- Distinguish recalled timing from verified evidence.
- Agree a corroboration step without assigning blame or a root cause.

Add a complication: A participant says the newer clinician's account should be discarded solely because they have less experience.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

Answer workshop

Try the tasks first. The model is one acceptable response to scenario 1, not a script to memorize. Assess your spoken version using the four criteria at the end.

01 | Shift Handoffs and Clinical Prioritization

Editing example: Please confirm which team has accepted the follow-up task.

The revised sentence asks for an identifiable receiving team instead of leaving responsibility with an unspecified 'someone'.

Check 1: B - Please confirm who will complete the outstanding check.

A: A written record alone does not identify who will act.

B: It asks for an identifiable person or team responsible for the task.

C: Mentioning a task does not show that anyone accepted it.

Check 2: A - So the result is pending and your team will follow up; is that correct?

A: This repeats the status and responsibility and invites correction.

B: It changes pending information into a final result.

C: Receiving a message is not the same as confirming its meaning.

Model response: The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

02 | Patient Assessment and Escalation

Editing example: Could you send the missing document and confirm when it will be available?

The revision names the document and requested response. It avoids expressions that may be unfamiliar or ambiguous for an international audience.

Check 1: A - Send it no later than Thursday.

A: 'By' sets the latest point, while 'after' refers to a later time.

B: "By Thursday" permits earlier delivery; it is not restricted to that day.

C: This interprets a deadline as a continuing activity, which would use "until".

Check 2: C - Could you confirm the document version needed for the review?

A: This checks importance rather than identifying the needed document version.

B: This checks receipt, not which version is required for the review.

C: The requested action and information are explicit.

Model response: I'm concerned because this is a change from the earlier observation. I need an assessment through our escalation process now. Here is what I observed and when it changed.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

03 | Medication Safety and Allergy Clarification

Editing example: Could you explain what this means?

After an introductory phrase such as 'Could you explain', the embedded question uses statement word order: "what this means".

Check 1: C - Could you confirm when the review starts?

A: Remove 'does' and use statement word order after 'confirm'.

B: Place the subject 'the review' before 'starts'.

C: The embedded question uses subject then verb: "the review starts".

Check 2: B - Has the report been approved, or is it ready for review?

A: This asks about the presenter, not the report's approval status.

B: This question distinguishes two meanings of 'ready' that affect the next action.

C: This asks about delivery timing, not whether approval has happened.

Model response: The recorded allergy information differs from what the patient has told me. Could we verify the details through the appropriate clinical process? I will state the discrepancy clearly rather than assume either account is complete.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

04 | Patient Education and Teach-Back

Editing example: We need to put the plan into practice.

The revised sentence names the action in familiar words. Specialist vocabulary is useful when it adds precision, but repeating abstract nouns can hide the meaning.

Check 1: B - The backlog is the work still waiting to be completed.

A: This describes capacity, not the work waiting to be completed.

B: This gives the meaning in familiar words.

C: Completed work is different from work still waiting.

Check 2: A - How would you describe the next step in your own words?

A: An open request lets the listener show their understanding.

B: Offering repetition may help, but it does not show how the listener understood the next step.

C: This invites an opinion on detail, not a demonstration of understanding.

Model response: I want to check that I explained this clearly. In your own words, how will you follow the instructions when you get home? We can go over any part that is unclear.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

05 | Interprofessional Rounds

Editing example: Before I record agreement, does anyone have a concern or need more time?

The revised question gives people a way to raise a concern. Silence can have several meanings, especially in unfamiliar or unequal-power situations.

Check 1: A - We haven't heard from the remote team yet; what is your view?

A: It invites participation without interpreting silence as consent.

B: Silence does not establish agreement.

C: Recording agreement without checking may misrepresent the team's view.

Check 2: C - Approval is pending; the sponsor will review the two options.

A: Discussion does not itself constitute approval.

B: Participation alone does not establish a decision.

C: This distinguishes discussion from approval and identifies the next reviewer.

Model response: Before we agree on timing, could we hear the mobility concern and the current assessment? Let's confirm the outstanding questions and who will address them in the care plan.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

06 | Documentation and Charting

Editing example: The pilot worked in the tested setting; performance elsewhere remains untested.

The revision states the finding and its scope. It avoids turning limited evidence into a universal claim.

Check 1: C - The change may have contributed to the delay.

A: 'Must have' expresses a strong inference that the evidence may not support.

B: 'Definitely' presents the cause as certain.

C: 'May have contributed' presents a possibility without claiming the cause is established.

Check 2: B - In this pilot, the observed result was ...

A: A pilot in one location does not establish a service-wide observation.

B: This locates the finding in the setting actually studied.

C: Similarity alone does not establish that the result will transfer.

Model response: Let's replace that label with observable information. Record what the patient said or did, the relevant context, the action taken, and the response, following the documentation process.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

07 | Difficult Families and Boundaries

Editing example: I'm sorry my message did not explain the timing clearly.

The revision takes responsibility for the message without blaming the listener. It identifies what needs to be corrected.

Check 1: B - I'm sorry my earlier wording was unclear.

A: This acknowledges a missed date, not the communication problem in the question.

B: This acknowledges the speaker's contribution to the problem.

C: This acknowledges duration, but does not take responsibility for the unclear wording.

Check 2: A - The corrected date and the current confirmation status.

A: A repair needs usable corrected information, including any uncertainty.

B: The reader still lacks the information they need.

C: An unsupported new promise can create a second misunderstanding.

Model response: I can hear that the lack of information is frustrating. Let me explain what I can clarify and arrange contact with the appropriate team member for the questions outside my role.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

08 | Safety Events and Just Culture

Editing example: The pilot worked in the tested setting; performance elsewhere remains untested.

The revision states the finding and its scope. It avoids turning limited evidence into a universal claim.

Check 1: A - The change may have contributed to the delay.

A: 'May have contributed' presents a possibility without claiming the cause is established.

B: 'Must have' expresses a strong inference that the evidence may not support.

C: 'Definitely' presents the cause as certain.

Check 2: C - In this pilot, the observed result was ...

A: Similarity alone does not establish that the result will transfer.

B: A pilot in one location does not establish a service-wide observation.

C: This locates the finding in the setting actually studied.

Model response: The report should describe the event and available observations. We have not established the contributing factors. I'll remove the unsupported judgment and use the reporting process to raise the concern.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-28. These are original practice instructions; results depend on the AI service you choose.

Practice grammar in context

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Nursing and Allied Health English

Professional audience: nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Lesson 01: Shift Handoffs and Clinical Prioritization

Original fictional case: In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

Language workshop: Transfer information and responsibility

Communication goal: Give a handoff that the receiving person can confirm.

Language guidance: State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Useful frames (complete the gaps with case facts): The current status is ...; the outstanding item is ... / The record shows ... / Please confirm who has accepted ...

Vocabulary: SBAR: Situation, Background, Assessment, and Recommendation or Request: a framework for organizing a clear handoff or concern. / acuity: The severity or complexity of a patient's condition and associated care needs. / pending lab: A requested laboratory test or result that is not yet available. / watcher: A local clinical label sometimes used for a patient needing closer attention; its criteria must be clarified locally.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Writing task: Write a 70-110 word message for the person who needs to act on this case. State the purpose, preserve the relevant facts, and make the next action or unresolved question clear. Use a subject line and an appropriate opening. Do not invent a deadline, finding, or approval.

Optional second-round challenge: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for comparison AFTER my attempt, not a response to give me first: The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

END REFERENCE

TASK: Grammar that changes the meaning

1. For a lesson, use its language workshop as the focus. For a dialogue, quote one actual sentence and select one useful grammar feature from it, such as question word order, tense, modality, conditionals or clause linking. Explain in no more than 90 words how this feature helps the speakers do their work. Keep the grammar target narrow.
2. Give two short contrasting sentences using this workplace context. Explain the difference in time, certainty, condition or politeness. Label invented examples and preserve the distinction between possible, planned and confirmed events. Describe context-dependent choices as choices, not universal rules.
3. Give me one editing or sentence-building task using a known case fact. Do not copy the supplied editing example or reveal its answer. Ask me to explain my intended meaning. Stop and wait.
4. After I answer, quote my wording, identify at most two issues, and give a brief explanation and one hint. Ask me to revise before offering a full corrected version. Then accept any accurate, natural alternative that serves the purpose.
5. Continue with two new tasks, one at a time: first guided, then an original response without a sentence frame. End with a two-sentence workplace message using the target feature and a compact self-check. Do not replace this sequence with a worksheet and answer key.

END PROMPT

Copy this prompt online or choose another lesson and support level

Get feedback on a draft

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Nursing and Allied Health English

Professional audience: nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Lesson 01: Shift Handoffs and Clinical Prioritization

Original fictional case: In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

Language workshop: Transfer information and responsibility

Communication goal: Give a handoff that the receiving person can confirm.

Language guidance: State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Useful frames (complete the gaps with case facts): The current status is ...; the outstanding item is ... / The record shows ... / Please confirm who has accepted ...

Vocabulary: SBAR: Situation, Background, Assessment, and Recommendation or Request: a framework for organizing a clear handoff or concern. / acuity: The severity or complexity of a patient's condition and associated care needs. / pending lab: A requested laboratory test or result that is not yet available. / watcher: A local clinical label sometimes used for a patient needing closer attention; its criteria must be clarified locally.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Writing task: Write a 70-110 word message for the person who needs to act on this case. State the purpose, preserve the relevant facts, and make the next action or unresolved question clear. Use a subject line and an appropriate opening. Do not invent a deadline, finding, or approval.

Optional second-round challenge: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for comparison AFTER my attempt, not a response to give me first: The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

END REFERENCE

TASK: Coach my workplace writing

1. Ask me to paste my own fictional or anonymized draft and identify its reader and purpose. If the reference gives a writing task, mention that task as the default. If it is a dialogue, suggest a brief follow-up message that records its outcome and open questions. Stop and wait. Do not write the message for me first.
2. Once I supply the draft, check it against the reference. If an ambiguity changes the meaning, ask one focused question before rewriting. Do not assume that the draft is correct evidence for a new deadline, approval, diagnosis, cost or commitment.
3. Give feedback in three parts: one effective phrase with a reason; up to three priority improvements, quoting my words; and one short revision task for me. Prioritize incorrect facts or unclear action before minor grammar. For each language correction, explain why it matters to this reader. Label optional stylistic alternatives separately and preserve my level of certainty and intended politeness.
4. Stop and wait for my revision. Then compare the two attempts, identify an improvement and offer one edited version that preserves my voice and purpose. Keep the requested length; if none is specified, use 70-110 words. Explain any meaningful change rather than silently making the message stronger or more certain.
5. Close with one phrase worth reusing and a short transfer task for a different fictional reader. Wait for my attempt before providing another model.

END PROMPT

Copy this prompt online or choose another lesson and support level